

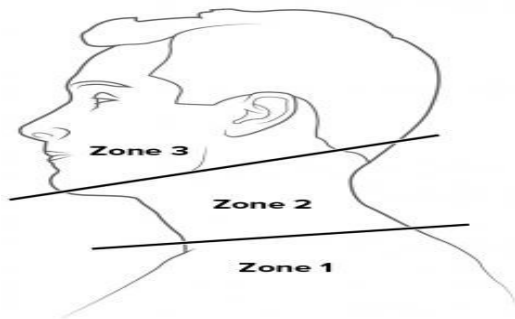
Neck Trauma

History

- Type of Injury / Mechanism? Time of injury?
- Blunt (eg. MVC) or penetrating (eg. GSW / stab) injury?
- If GSW, what was caliber of bullet?
- Strangulation / Hanging? (Considered blunt trauma)

Key Concepts

- Evaluate for any penetrating mechanism of injury
- Any violation of the skin in Zone1/Zone2 can be a surgical emergency
- Intubate early for signs of stridor or neck swelling.
- Immobilize penetrating objects in place



MARCHES Protocol

- **Massive Bleeding control** - tourniquet/wound packing
- **Airway** – NPA/OPA/Cricothyrotomy
- **Respiratory** – decompress chest if tension pneumothorax suspected, occlusive dressing for open pneumothoraces
- **Circulation**- IV/IO, pelvic binder, reassess tourniquet/ wound packing
- **Hypothermia care/Hike** vs helicopter (consider evac options)
- **Eye injuries** – cover with rigid shield and no pressure on the eye
- **Spinal restriction** if indicated

Physical

- Fully expose patient and do thorough exam
- Vital signs? – Hypotension? Hypoxia? Bradycardia?
- Mental Status Altered? Signs of head injury?
- Which Zone is injury in?
- Respiratory difficulty? Is airway patent? Any signs of pneumothorax?
- Bleeding? Where? How much? Arterial? Expanding hematoma?
- Neurologic findings – Paralysis? Incontinence? Priapism?

Zones of the Neck (Important for describing neck injuries)

- **Zone 1** – Base of the neck, from level of clavicles and sternal notch to cricoid cartilage
- **Zone 2** – Mid-portion of neck, from cricoid cartilage to angle of mandible
- **Zone 3** – Superior aspect of neck from angle of mandible to base of skull

EMT/AEMT

- Stop **Massive Bleeding** as first action
 - Pressure dressing with firm direct manual pressure
- Establish Simple **Airway** with NPA or OPA and suction secretions if needed
- Support **Respirations**
 - BVM or apply Oxygen
- Support **Circulation**
 - Reassess wounds and do rapid head to toe trauma survey
- Assess level of Consciousness with GCS or AVPU tool
- Protect from **Hypothermia**
- **Eye injuries** -cover with rigid shield and no pressure on the eye
- **Spinal motion restriction** if indicated
- Prepare for transport or move patient towards Medic Unit

Paramedic

- Assess BLS **MARCHES** interventions
- **Limit scene time** with only EMT level skills and lifesaving interventions to be done prior to load
- Transport with head elevated 30 degrees if possible
- **Airway**-Consider advanced airway management
- **Respiration**- ensure adequate ventilation
 - Capnography is required for all patients being ventilated or with altered LOC
- **Circulation**- Start IV/IO and administer fluids according to Trauma General Patient Care Guideline
- **Tranexamic acid (TXA)** as indicated per TXA Guideline
- Treat Pain, Seizures, Anxiety/Behavioral Emergencies as per appropriate Guideline